

Procedure Date:
Procedure Physician:
Attending Physician/Copies To:

Carcinoma, infiltrating ductal mixed with other type
Site: Breast, NOS C50.9 8523/3

1/21/11
hr

PATIENT HISTORY:

* DATE OF LMP: *

DATE OF LAST DELIVERY: *

PRE-OP DIAGNOSIS: RT BREAST CANCER

POST-OP DIAGNOSIS: SAME

OPERATIVE PROCEDURE: RT SEG MASTECTOMY, RT CX NODE DISSECTION

CLINICAL HISTORY: *

MATERIAL SUBMITTED: A) (RIGHT) BREAST BIOPSY/MASS/SEGMENTAL MASTECTOMY, PROCUREMENT BY SURGICAL PROCEDURE
B) RIGHT AXILLARY CONTENTS

INTRAOPERATIVE CONSULTATION:

FROZEN SECTION: Right breast: 2 masses, 1.0 by 1.5 by 1.2 cm 0.4 cm from medial margin. Other 1.5 by 1.0 by 1.5 cm 0.5 cm from superior margin.

Frozen section diagnosis: (tumor near medial margin): Infiltrating carcinoma with features of ductal and lobular carcinoma.

FINAL DIAGNOSIS:

PREVIOUS REPORT: ---

A&AFS) RIGHT BREAST SEGMENTAL MASTECTOMY:

- MULTIFOCAL INFILTRATING DUCT CARCINOMA WITH LOBULAR AND CLEAR CELL FEATURES 1.5 BY 1.5 BY 1.0 CM AND 1.5 BY 1.2 BY 1.0 CM, INTERMEDIATE NUCLEAR GRADE, POOR ARCHITECTURAL GRADE, WITH ASSOCIATED MICROCALCIFICATIONS

- FOCAL DUCTAL CARCINOMA IN SITU, NON-COMEDO TYPE, COMPRISING <5% OF TUMOR

- SURGICAL MARGINS NEGATIVE FOR TUMOR BUT TUMOR CLOSELY APPROACHES MEDIAL MARGIN (WITHIN 0.2 CM)

B) RIGHT AXILLARY CONTENTS:

- ONE (1) OF FIFTEEN (15) NODES POSITIVE FOR TUMOR AND SHOWING EXTRA CAPSULAR EXTENSION

S U P P L E M E N T A L R E P O R T

ER/PR

My signature below is attestation that I have reviewed all slides and agree with the findings as noted below.

IMMUNOPEROXIDASE IDENTIFICATION OF (ESTROGEN AND PROGESTERONE RECEPTORS) IS CARRIED OUT ON SLIDE A1. DISTINCT INTRANUCLEAR STAINING IS IDENTIFIED FOR BOTH ESTROGEN RECEPTOR (90%) AND PROGESTERONE RECEPTOR (90%). THEREFORE, BOTH ARE INTERPRETED AS POSITIVE.

Pathologist

S U P P L E M E N T A L R E P O R T

HER-2/NEU

My signature below is attestation that I have reviewed all slides and agree with the findings as noted below.

c-erbB2 (HER-2/NEU) IMMUNOSTAINING IS CARRIED OUT ON BLOCK A1 (BREAST CANCER) USING A 1:300 DILUTION OF DAKO'S POLYCLONAL ANTIBODY A485 (DIRECTED AGAINST THE INTRACELLULAR DOMAIN OF c-erbB2) WITHOUT ANTIGEN RETRIEVAL. NO DISTINCT COMPLETE MEMBRANE STAINING IS IDENTIFIED. THEREFORE, c-erbB2 (HER-2/NEU) IS INTERPRETED AS NEGATIVE (SCORE 0).

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	MS	MS
Date Reviewed	1/21/11	